CONSULT FOR AFRICA
Build. Transform. Grow.

PREPARED FOR CARBON

Pilot terms for hospital receivables

Our answers to your five points, how each one works, and what the hospital pays in total.

THE TERMS

60%

of the accepted claim, advanced

2.5%

a month to Carbon, 60 days minimum

1%

of the advance to us, once, paid last

FOR	FROM	DATE
Chijioke Dozie, Carbon	Dr Debo Odulana	12 October 2026

Yes to all five, with one change from us that makes them easier for a hospital to accept.

Your words in italics, our answer under them. The pages that follow take each point in turn.

Recourse	*Full recourse to the hospital on every advance, not only rejections it controlled.* Agreed. The hospital repays any advance its HMO has not settled by the due date, whatever the reason.
Order of payment	*Your fee on a hospital's book comes out after Carbon has been repaid on it.* Agreed. Our 1% is withheld at disbursement and released only while nothing on that book is overdue.
HMOs and the account	*Acknowledgment letters from the top HMOs, and all HMO receipts into the Carbon account.* Agreed, as the default. I will take the letters to the HMOs myself, with the account openings.
Total cost	*What you charge the hospital for recovery. I want to see what the hospital pays in total.* 6% of the advance if the HMO pays inside 60 days, 11% at 120 days. Nothing else is charged.
Payors	*Private HMOs only, no government or military payers, and add diagnostic centres.* Agreed. Diagnostic centres join the first batch alongside the hospitals.

Our change: we only fund claims the HMO has already accepted, and never more than 60% of the accepted value.

Full recourse on every advance, and Carbon repaid before we are.

Due date	Set per HMO from its payment record, 120 days after disbursement at most.
If the HMO is late	Carbon recovers the advance and its interest from the hospital's other HMO receipts in the collection account, oldest overdue first. We keep pursuing the HMO for the hospital.
New draws	Stop for that hospital while any of its advances is overdue.
Our fee	Withheld at disbursement. Released only when the advance is repaid and nothing on the hospital's book is overdue.
Why overdue, not outstanding	A revolving book is never at zero, so a test of nothing outstanding would never be met.

WHEN AN HMO PAYS

1. **Carbon**
   principal, and interest for any days past the minimum
2. **Overdue advances**
   on the same hospital's book, oldest first
3. **Our 1%**
   released from what was withheld, if nothing is overdue
4. **The hospital**
   everything else, within 48 hours

The top HMOs sign before a hospital draws, and every HMO payment goes through Carbon.

THE ACKNOWLEDGMENT LETTER

Each HMO confirms, for each hospital:

1. the amount it has accepted on each claim;
2. that it pays into the hospital's collection account at Carbon;
3. that it will not set off older balances against funded claims without notice.

I will take the letter to the HMOs myself, alongside the account openings.

THE COLLECTION ACCOUNT

Every HMO receipt, funded or not.

HMOs pay in batches across many claims, so routing everything through one account is also the only clean way to reconcile. The split is recorded claim by claim the day the money lands.

Whatever is not owed to Carbon, or to us under the waterfall, goes to the hospital's operating account within 48 hours, so it keeps its working capital.

60%
of a hospital's HMO billing must be on signed HMOs before it draws

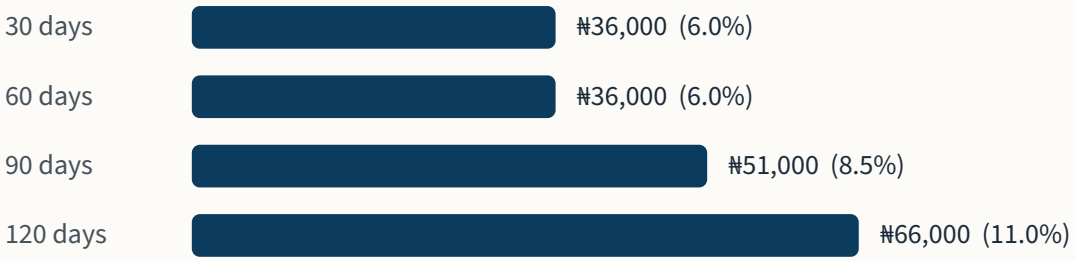
Signed only
claims are funded only on the HMOs that have signed

48 hours
for the hospital's share to leave the collection account

On every ₦1,000,000 the HMO accepts, the hospital pays 6% of the advance if the HMO pays within 60 days, and 11% at 120.

Advance, 60% of the accepted claim	₦600,000
Carbon's interest for the first 60 days, 2.5% a month	less ₦30,000 taken at disbursement
Our fee, 1% of the advance	less ₦6,000 withheld, paid after Carbon
Reaches the hospital on day one	₦564,000 within 48 hours

TOTAL COST BY THE DAY THE HMO PAYS



ON YOUR RATE POINT

Interest now runs daily on the days the money is out, with the HMO's expected time to pay as the minimum. Days beyond it come out of the HMO's payment.

30%+

a year to Carbon on the days its money is out, never less, and more when an HMO pays early. The CBN policy rate is 23%.

Recovery of claims an HMO has not yet accepted is separate work, agreed with the hospital and paid behind Carbon. It never touches the funded book.

Private HMOs only, with diagnostic centres alongside the hospitals in the first batch.

FUNDED

Private HMOs

Claims each HMO has accepted, from HMOs that have signed the letter for that hospital.

NOT FUNDED

Every other payor

NHIA, state schemes, military and corporate accounts. Their claims can still come through the desk for recovery, but are never advanced against.

Hospitals	Private hospitals with a meaningful HMO book. Each starts with a free review of its outstanding claims before anything is signed.
Diagnostic centres	Small, frequent claims priced off standard tariffs, which suit a first tranche. Same review, same rules, same letters.

If these land, free reviews start this month and the first tranche follows.

WEEKS 0 TO 2

Terms

Facility agreement signed by Carbon, the hospital and us. Collection accounts opened.

WEEKS 2 TO 4

HMO letters and reviews

Letters taken to the top HMOs. Free reviews of each facility's outstanding claims.

WEEK 4

First tranche

To the first facility whose HMOs have signed, against claims they have accepted.

DAY 90

Review

Rate, recourse and facility sizes reset on the pilot's own numbers.

Agreed first	The loss rate that stops the pilot, and the payment record that lifts an HMO's advance or a facility's limit.
Already built	Our desk already records HMO acceptance and the letters, fixes each advance's terms at disbursement, splits every receipt and produces the book for Carbon each week.

Three things from Carbon, and we can start.

1

Credit criteria

and the facility limit you want for each hospital and diagnostic centre.

2

Letter wording

for the HMO acknowledgment, from your counsel, so the HMOs see one version.

3

Data format

for the claim-level book and receipts: a weekly file now, a feed later.

Debo

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